oxygen to Delhi daily, it may consider initiating contempt proceedings.[66] In fact, the Delhi High Court directed the Centre to show cause as to why contempt proceedings should not be initiated against it for failing to comply with its order on the supply of oxygen to Delhi. Meanwhile, Delhi was not able to lift even its allocated quantity of oxygen and Kejriwal, in a video conference, announced that they would import 18 cryogenic tankers from Bangkok and 21 ready-to-use oxygen plants from France with the help of the Indian Air Force, if the Centre permits.[67] Immediately, the GoI agreed to let the Delhi government use the services of the Indian Air Force for the same. However, six months in, the Delhi government hasn't been able to import a single tanker or oxygen plant. On the flip side, many states such as Andhra Pradesh, Telangana and Odisha had made arrangements beforehand in conjunction with their top oxygen suppliers and imported tankers in a long-lease mechanism.

The Centre then moved the Supreme Court against Delhi High Court's show-cause notice of contempt[68], which then came down hard on them holding them in contempt if they failed to provide Delhi's demanded 700 MT oxygen every day—even as they took cognisance of the fact that Delhi may have mismanaged oxygen supply and distribution. They came down heavily on the Delhi government stating, 'We want you to convey a message to the highest authority that there should be a sense of cooperation with the Centre in this humanitarian crisis. We don't want political bickering at the time of crisis. Politics is at the time of elections. There has to be a sense of cooperation. A spirit of dialogue. Saving lives is the priority.'[69] The Court constituted a 12-member National Task Force, convened by the Cabinet secretary, to formulate a methodology to achieve the goal of scientific, rational and equitable distribution of oxygen to the states.[70] Inspired by Delhi's demand, several other opposition-run states also began to flag oxygen shortages. Oxygen allocation made to states by the Centre was based on the formula which took into account current cases along with availability of hospital infrastructure including ICU beds. If states did not reveal real case numbers, naturally allocations would not be sufficient. Once again, quoting MCGM Commissioner Iqbal Chahal becomes pertinent to make my point clear. In an interview, he was asked who was to blame for the oxygen crisis plaguing the country to which he replied,